

UUID: BFC4CD62-F17E-4F08-8766-389A882167FE
TCGA-EM-A3FL-01A-PR

Redacted



Surgical Pathology Consultation Report

Patient Name:

MRN:

DOB:

Gender:

Ordering MD:

Copy To:

Service:

Visit #:

Location:

Facility:

Accession #:

Collected:

Received:

Reported:

Specimen(s) Received

1. Parathyroid: left upper parathyroid qs
2. left hemi- thyroid stitch upper pole

Diagnosis

1. No pathological diagnosis: Parathyroid (left upper) biopsy
2. Multifocal papillary carcinoma, dominant follicular variant, 1.5 cm; arising in follicular nodular disease and chronic lymphocytic thyroiditis: Thyroid (left) hemithyroidectomy specimen

Comment

There is a unifocal 1.5 cm papillary carcinoma transformation in the dominant lesion.

Synoptic Data

Procedure: Left hemithyroidectomy
Received: Fresh
Specimen Integrity: Intact
Specimen Size: Left lobe:
4.3 cm
3.3 cm
2.3 cm
Isthmus +/- pyramidal lobe:
3.0 cm
1.7 cm
1.2 cm
Specimen Weight: 23.5 g
Tumor Focality: Multifocal, ipsilateral
----- DOMINANT TUMOR -----
Tumor Laterality: Left lobe
Tumor Size: Greatest dimension: 1.5cm
Histologic Type: Papillary carcinoma, follicular variant
Papillary carcinoma, oncocyctic or oxyphilic variant
Follicular architecture
Classical cytomorphology
Oncocyctic or oxyphilic cytomorphology
Histologic Grade: Not applicable

ICD-O-3

Carcinoma, papillary, follicular variant

8340/3

Site: thyroid, NOS

C73.9

hw 6/8/12

Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
HIPAA Discrepancy		X
Prior Malignancy History		X
Dual/Synchronous Primary Noted		X
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewed by:	hw 6/8/12	11/2/11

Surgical Pathology Consultation Report

Margins:	Margins uninvolved by carcinoma
Tumor Capsule:	None
Tumor Capsular Invasion:	Not identified
Lymph-Vascular Invasion:	Not identified
Perineural Invasion:	Not identified
Extrathyroidal Extension:	Not identified
----- SECOND TUMOR -----	
Tumor Laterality:	Left lobe Isthmus
Tumor Size:	Greatest dimension: 0.7cm Additional dimensions: 0.2 cm
Histologic Type:	Papillary carcinoma, follicular variant Papillary carcinoma, microcarcinoma (occult, small or microscopic) variant Papillary carcinoma, oncocytic or oxyphilic variant Follicular architecture Classical cytomorphology Oncocytic or oxyphilic cytomorphology
Histologic Grade:	Not applicable
Margins:	Margins uninvolved by carcinoma
Tumor Capsule:	None
Tumor Capsular Invasion:	Not identified
Lymph-Vascular Invasion:	Not identified
Perineural Invasion:	Not identified
Extrathyroidal Extension:	Not identified
TNM Descriptors:	m (multiple primary tumors)
Primary Tumor (pT):	pT1b: Tumor more than 1 cm but not more than 2 cm in greatest dimension, limited to the thyroid
Regional Lymph Nodes (pN):	pNX: Regional lymph nodes cannot be assigned Number of regional lymph nodes examined: 0 Number of regional lymph nodes involved: 0
Distant Metastasis (pM):	Not applicable
Additional Pathologic Findings:	Adenomatoid nodule(s) or Nodular follicular disease Thyroiditis, other: chronic lymphocytic thyroiditis
Ancillary Studies:	Type: IHC Results: The 1.5 cm focus (slide no 2E) is negative for Galectin-3 and is focally positive for cytokeratin 19. Few cells are positive for HBME-1 with an apical and membranous pattern.

*Pathologic Staging is based on AJCC/UICC TNM, Edition

Electronically verified by:

Gross Description

1. The specimen labeled with the patient's name and as "Parathyroid: Left upper parathyroid QS" consists of a piece of reddish tissue that weighs 0.007 g and measures 0.4 x 0.2 x 0.1 cm. It is frozen for intraoperative consultation.

1A frozen section block resubmitted

2. The specimen labeled with the patient's name and as "left hemi-thyroid stitch upper pole", consists of a left hemithyroidectomy specimen that weighs 23.5 g. The lobe measures 4.3 cm SI x 3.3 cm ML x 2.4 cm AP and the isthmus measures 3.0 cm SI x 1.7 cm ML x 1.2 cm AP. No parathyroid glands and no lymph nodes are identified grossly. The external surface is painted with blue dye. The upper to lower pole of the lobe contains a well delineated nodule that measures 4.1 cm SI x 3.1 cm ML x 2.3 cm AP. The remainder of the thyroid tissue is unremarkable. Sections of nodule and normal tissue are stored frozen. The remainder of the specimen is submitted as follows:

[REDACTED] **Surgical Pathology Consultation Report** [REDACTED]

2A-J lobe, superior to inferior
2K-M isthmus

Quick Section Diagnosis

QS-1A: Parathyroid tissue identified: Parathyroid (left upper) biopsy. Dr.

Microscopic Description

The 1.5 cm focus (slide no 2E) is negative for Galectin-3 and is focally positive for cytokeratin 19. Few cells are positive for HBME-1 with an apical and membranous pattern. The rest of dominant lesion is negative for HBME-1, cytokeratin 19 and Galectin-3.
